PREOPERATIVE DIAGNOSIS: Severe chronic pelvic pain, dyspareunia, and dysmenorrhea refractory to oral contraceptives and NSAIDs; suspected pelvic endometriosis.

POSTOPERATIVE DIAGNOSIS: Stage III (moderate) pelvic endometriosis with bilateral ovarian endometriomas and posterior cul-de-sac obliteration.

PROCEDURE: Diagnostic laparoscopy, excision of peritoneal endometriosis implants, bilateral ovarian cystectomy, and extensive lysis of pelvic adhesions.

PROCEDURE NARRATIVE: Under general endotracheal anesthesia, the patient was prepared in the lithotomy position. A 10 mm umbilical trocar was inserted, and pelvic pneumoperitoneum was established. Inspection of the pelvis revealed extensive powder-burn and dark flame-like endometriotic peritoneal implants across both uterosacral ligaments, the pouch of Douglas, and the right ovarian fossa. Both ovaries were adherent to the posterior leaf of the broad ligament. The right ovary harbored a 4 cm endometrioma, and the left ovary harbored a 3 cm endometrioma.

Using blunt and sharp laparoscopic dissection with hydrodissection, the ovaries were mobilized off the pelvic sidewalls. The endometrioma capsules were opened, draining characteristic thick, dark-brown 'chocolate' fluid, which was thoroughly aspirated and irrigated. The cyst wall linings were dissected from the normal ovarian parenchyma using dual-grasping traction and stripped in their entirety. Hemostasis was secured with low-power bipolar electrocautery to preserve ovarian reserve.

Attention was then turned to the peritoneal lesions. Using sharp cold scissors and laparoscopic argon beam coagulation, all visible peritoneal endometriotic deposits along the uterosacral ligaments and pelvic sidewalls were radically excised down to healthy retroperitoneal fat, mobilizing the ureters bilaterally to ensure clear margins. An anti-adhesion barrier (Interceed) was placed over the denuded pelvic surfaces. Copious saline irrigation was carried out until clear.